جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : مرفت مصطفى على عبدالله

المحافظة : الجيزه **الحي / المدينة :** الاخصاص **العنوان :** 5 شايوب - الاندلس الهرم -الجيزه

رقم البطاقة : 26103160101565 **رقم التليفون :** 01001439768 **تاريخ تسجيل الإستمارة :** 21-10-2026

العمر(تاريخ الميلاد) : 16-03-1961 **المهنة :** بدون عمل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي
التوقيع:(----------)

المقر بما فيه
التوقيع:(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: مرفت مصطفى على عبدالله
National ID: 26103160101565

GUSTAVE ROUSSY INTERNATIONAL EGYPT

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

[[DOCTYPE:MRI]]
M.R.I

* For comparison with this report as patient is on ttt (Ibrance)

Baheya

T 16602
F 16602
W info@baheya.org
E www.baheya.org
شارع علوية أول شارع الهرم بجوار داري , الجيزة , مصر 4
شارع علوية أول شارع الهرم بجوار داري , الجيزة , مصر 4
1 Giza
Giza, Egypt

مركز بهية للاكتشاف المبكر
وعلاج سرطان السيدات

Patient Name: **MERVAT MOUSTAFA ALI ABDALLA**	Gender: **Female**
MRN: **2020142884**	Ordered By: **marwa mahmoud abdelrahman mohamed selim**
DOB: **16/03/1961** Age: **65 Y 7 M 3 D**	Order ID: **4038471**

NM BONE SCAN

Procedure Date: 14/10/2026 03:34:00 PM

Report:

Report:
99mTc-MDP BONE SCAN

Desired Information: -
skeletal survey.

Technique:
Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

Report:

Compared to previous bone scan dated 11/4/2026, the current study revealed:-
Resolution of the previously regressed LV3 lesion.
No newly developed bone lesions.
Degenerative changes are seen over dorso-lumbar vertebrae.
Both kidneys are normally visualized.

Conclusion:

Follow up bone scan shows resolution of the LV3 lesion.

Approved by: Huda Fathy Elsayed Abdulhalim
Date:14/10/2026 Time:15:05

Reported and electronically endorsed by:
Huda Fathy Elsayed Abdulhalim

Reported on: 14/10/2026 03:17:00 PM

[[DOCTYPE:MRI]]
M.R.I

بهية
مركز بهية للاكتشاف المبكر وعلاج سرطان السيدات

Baheya

T 16602
F 16602
W info@baheya.org
E www.baheya.org
شارع علوية أول شارع الهرم بجوار دارى , الجيزة , مصر 4
شارع علوية أول شارع الهرم بجوار دارى , الجيزة , مصر 4
1 Giza
Giza, Egypt

Patient Name: **MERVAT MOUSTAFA ALI ABDALLA**	Gender: **Female**
MRN: **2020142884**	Ordered By: **marwa mahmoud abdelrahman mohamed selim**
DOB: **16/03/1961** Age: **65 Y 7 M 3 D**	Order ID: **4038468**

2. Left femoral neck irregular cortical outlines.
3. Mild dorsolumbar spondylosis.
4. Bilateral sacroiliitis is noted
5. Still seen bilateral iliac, sacral and right femoral head/neck sclerotic foci.

Opinion:
- Stable post-operative status of right mastectomy, with no evidence of local recurrent mass or operative bed collection on this non-contrast study.
- Stationary few subcentimetric right axillary lymph nodes.
- Stable few tiny bilateral pulmonary nodules.
- No definite hepatic focal lesion or abdominopelvic metastatic disease detected on this non-contrast CT study.
- Stable multiple sclerotic osseous foci involving L4 vertebral body, bilateral iliac bones, sacrum and right femoral head/neck.
- Stable left femoral neck cortical irregularity.

Approved by: Mahmoud Ibrahim, MD
Date:14/10/2026 Time:10:36

Reported and electronically endorsed by:
Mahmoud Ibrahiem Ibrahiem Ghazy

Reported on: 15/10/2026 02:49:00 PM

[[DOCTYPE:CT_SCAN]]
CT Scan

بهية
مركز بهية للاكتشاف المبكر وعلاج سرطان السيدات

Baheya
T 16002
F 16002
W info@baheya.org
E www.baheya.org
شارع طرية أول شارع الهرم بجوار ناري , الجيزة , مصر 4
شارع طرية أول شارع الهرم بجوار ناري , الجيزة , مصر 4
1 Giza
Giza, Egypt

Patient Name: **MERVAT MOUSTAFA ALI ABDALLA**	Gender: **Female**
MRN: **2020142884**	Ordered By: **marwa mahmoud abdelrahman mohamed selim**
DOB: **16/03/1961** Age: **65 Y 7 M 3 D**	Order ID: **4038468**

CT ABDOMEN AND PELVIS

Procedure Date: 14/10/2026 10:33:00 AM

Report:

Report:
NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS

**A known case of breast cancer followed by surgical intervention.
**As compared to the previous study dated 25/06/2026, the current study revealed:

1- CHEST:
- Still seen clear right mastectomy operative bed with no sizable masses or collections seen.
- No appreciable changes regarding the previously seen:
1. Few right axillary Subcentemetric level I lymph nodes
2. Few bilateral tiny pulmonary nodules; the largest is still seen at the middle lobe measuring 4 mm in diameter.
3. Middle lung lobe, lingular and left lower lung lobe parenchymal bands.
4. Prominent right pericardial fat pad.
5. Prominent mediastinal lymph nodes.
- No gross cardiomegaly.
- Mildly patulous thoracic esophagus.
- Aberrant right subclavian artery (variant).
- No pleural or pericardial effusion detected.
- Still noted left thyroid lobe small calcified nodules, for ultrasound correlation.

2- ABDOMEN AND PELVIS:
- The liver is mildly enlarged in size. No definite hepatic focal lesions on non contrast basis.
- No intra-hepatic or extra-hepatic biliary radicles dilatation on non contrast basis.
- Normal non contrast appearance of the pancreas, spleen, kidneys and adrenals.
- Still noted:
1. Bulky uterus.
2. Small fat containing umbilical hernia.
3. Mild aortic atheromatous calcification.
- Empty urinary bladder.
- No abdominal or pelvic masses or collections.
The revised bone window revealed:
- Still seen:
1. Stable L4 upper vertebral end plate sclerotic lesion.



٤

National ID – Front
[[DOCTYPE:ID_FRONT]]



National ID – Back
[[DOCTYPE:ID_BACK]]

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : مرفت مصطفى على عبدالله Sex: 3- Medical number: 83209855 Female Old Med : Old Med : 1702-2024
2-ID number: 26103160101565 Patient Age : 63 Y

MDT

Family history	Family History	Phone	Phone 01001439768	BMI	BMI	REASON OF MDT	REASON OF MDT
marital status	Marital Status	NRS Score	NRS Score	GROUP	GROUP	Date of MDT	Date of MDT 07/07/2024
Co morbidity	Co Morbidity htn,cardiac	Menopausal status	Menopausal Status	Initial STAGE	Initial STAGE		

Clinical data

Breast Imaging :

Mets Workup :

BX	ER	PR	Her 2 Neu	SISH	Ki67
BX COMMENT: multifocal mixed IDC & ILC	8/8 ER COMMENT:	3/3 PR COMMENT:	Her 2 Neu COMMENT: -ve	SISH COMMENT :	40

T	N	M

Sonomammography

PATIENT HISTORY

7/2021:rt simple m,astetcomy + SLN
5/2022:EC x 4 + anastrazole 26/6/2021 till 5/2024
3/2024:bone mets

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

* For PAN CTs with contrast (at BAHEYA) baseline
* Bone Scan
*Start CD K 4/6

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د/ هشام هرت د/ رضوى صبره	RADIOLOGY	RADIOLOGY د/ شريف سالم
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY Dr Sh/ Sh
Psychiatric	Psychiatric	Nutrition	Nutrition

Patient Name	: الأستاذه / مرفت مصطفى على عبدالله	Request No.	: 13002150
Patient No.	: 26103160101565	Request Date	: 29/05/2024
Path No.	: SR-690/24	Verification Date	: 03/06/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 63 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: A case of metastatic breast cancer to bone.
Nature of Specimen: Slide revision.

Gross Examination:
- One paraffin block labeled by (6565-20) was received.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+1).
* DCIS: Negative.
* Angioinvasion: Negative.
* Perineural invasion: Negative.

Diagnosis:
Metastatic breast cancer, slide revision:
Invasive duct carcinoma G II.
- For further IHC (ER, PR, Her2, Ki67).

Signature
Prof. Dr. Lubna Omar.
Dr. Menna Nabil.

Verified by :
DR.Menna Nabil

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / مرفت مصطفى على عبدالله	Request No.	: 13002150
Patient No.	: 26103160101565	Request Date	: 29/05/2024
Path No.	: SR-690/24	Verification Date	: 24/06/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 63 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:
Metastatic breast cancer to bone.

Nature of Specimen:
Block revision.

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:
* IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 80% of tumor cells (score 8/8).
- PR: Weak nuclear reaction in 9% of tumor cells (score 3/8).
- Her2: No membrane reaction in tumor cells (score 0).
- Ki67: 19 % nuclear reaction of tumor cells.

Conclusion:
Metastatic breast cancer, block revision:
Invasive duct carcinoma, Grade II.
IPT:
- ER:Positive (score 8/8).
- PR:Positive(score 3/8).
- Her2:Negative (score 0).
- Ki67: Positive in 19% of tumor cells.

Verified by :
dr.esraa magdy

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name	الأستاذه / مرفت مصطفى على عبدالله	Request No.	13002150
Patient No.	26103160101565	Request Date	29/05/2024
Path No.	SR-690/24	Verification Date	24/06/2024
Sex	Female	Financial Category	اورام دار السلام - نفقة الدولة
Age	63 Y	Department	Out Patient
Referreing Doc.		Registered by	Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Prof.Lubna Omar
Dr.Menna Nabil

Verified by :
dr.esraa magdy

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

مركز بهية للاكتشاف المبكر
وعلاج سرطان الثدي

Baheya

T 16602
F 16602
W info@baheya.org
E www.baheya.org
شارع علي أول شارع الهرم بعد دار ي ، الجيزة ، مصر 4
شارع علي أول شارع الهرم بعد دار ي ، الجيزة ، مصر 4
1 Giza
Giza, Egypt

Patient Name:	**MERVAT MOUSTAFA ALI ABDALLA**	Gender:	**Female**
MRN:	**2020142884**	Ordered By:	**maha alaaeldin abdelfatah abdallah**
DOB:	**16/03/1961** Age: **65 Y 1 M 2 D**	Order ID:	**3818044**

NM BONE SCAN

Procedure Date: 11/04/2026 09:46:00 AM

Report:

Report:
99mTc-MDP BONE SCAN

Desired Information: -
skeletal survey.

Technique:
Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP :

Report:

Compared to previous bone scan dated 20/9/2025, the current study revealed:-
Further regression of the warm uptake at LV3.
No newly developed bone lesions.
Degenerative changes are seen over dorso-lumbar vertebrae.
Arthritic changes are seen over both knees.
Both kidneys are normally visualized.

Conclusion:
Follow up bone scan shows further regression with no definite evidence of metastatic osseous deposits.

Approved by: Prof. Huda Fathy, Nuclear Medicine Consultant
Date:11/04/2026 Time:15:10

Reported and electronically endorsed by:
Huda Fathy Elsayed Abdulhalim

Reported on: 11/04/2026 03:12:00 PM

Bone Scan
[[DOCTYPE:BONE_SCAN]]

BAHEYA
Radiology Department

بهية
قسم الأشعة التشخيصية

PATIENT NAME: MERVAT MOUSTAFA ALI ABDALLA **GENDER:** Female **DATE :** 20-Sep-2025 10:41 AM	**PATIENT ID:** 2020142884 **AGE:** 64 y **ORDER:** 3454363

99MTC-MDP BONE SCAN

Desired Information: -
skeletal survey.

Technique:
Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

Report:

Compared to previous bone scan dated 23/2/2025, the current study revealed:-
Further regression of the warm uptake at LV3.
No newly developed bone lesions.
Degenerative changes are seen over dorso-lumbar vertebrae.
Arthritic changes are seen over both knees.
Both kidneys are normally visualized.

Conclusion:

Follow up bone scan shows further regression of the LV3 uptake.
No newly developed bone lesions.

Reported By,

Prof. Huda Fathy, Nuclear Medicine Consultant **20-Sep-2025 02:28 PM**

www.Baheya.org 16602
مركز بهية الهرم
٨ شارع علوية أول شارع الهرم بجوار نادي الجزيرة
مستشفى بهية الشيخ زايد
٤ الحي الأول - الشيخ زايد أمام كلية الهندسة

Bone Scan
[[DOCTYPE:BONE_SCAN]]

بهية
مركز بهية للاكتشاف المبكر
وعلاج سرطان السيدات

Bahaya
T 16023
F 16023
W info@bahaya.org
E www.bahaya.org
4 شارع بهية أول شارع الهرم بعد نادي الجزيرة، مصر
4 شارع بهية أول شارع الهرم بعد نادي الجزيرة، مصر
1 Giza
Giza, Egypt

Patient Name:	**MERVAT MOUSTAFA ALI ABDALLA**	Gender:	**Female**
MRN:	**2020142884**	Ordered By:	**Eman Sobhy Adrawos Mahgoub**
DOB:	**16/03/1961** Age: **65 Y 0 M 21 D**	Order ID:	**3723390**

CT ABDOMEN AND PELVIS

Procedure Date: 02/04/2026 09:56:00 AM

Report:

Report:
NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS

**Reported case of breast cancer underwent surgical intervention.
**Reviewing the previous study dated January 2026, the current study shows:

1- CHEST:
- Evidence of right mastectomy with still no sizable operative bed recurrent lesions. Few right axillary subcentemetric level I lymph nodes are still seen, the largest measures 5X3 mm.
- No gross interval changes regarding the previously noted:
1. Few bilateral prominent supraclavicular subcentimetric lymph nodes.
2. Few bilateral tiny pulmonary nodules; the largest is still seen at the middle lobe, still measures about 0.5 cm in diameter.
3. Middle lung lobe and lingular parenchymal bands.
4. Prominent right pericardial fat pad.
5. Prominent mediastinal lymph nodes.
- No gross cardiomegaly.
- Mildly patulous thoracic esophagus.
- Aberrant right subclavian artery (variant).
- No pleural or pericardial effusion detected.
- Still noted left thyroid lobe small calcified nodules

2- ABDOMEN AND PELVIS:
- The liver is mildly enlarged in size. No definite hepatic focal lesions on non contrast basis.
- No intra-hepatic or extra-hepatic biliary radicles dilatation on non contrast basis.
- Normal non contrast appearance of the pancreas, spleen, kidneys and adrenals.
- Still noted:
1. Bulky uterus, for US assessment if needed.
2. Small fat containing umbilical hernia.
3. Mild aortic atheromatous calcification.
- Regular filling of the urinary bladder showing smooth outline with no sizable masses or diverticular outpouching.
- No abdominal or pelvic masses or collections.
The revised bone window revealed:

CT Scan
[[DOCTYPE:CT_SCAN]]

مركز بهية للاكتشاف المبكر
وعلاج سرطان الثدي

Bahaya

T 19802
F 16602
W info@bahaya.org
E www.bahaya.org
شارع عزبة زان شارع الهرم بجوار دارلي ، الجيزة ، مصر 4
شارع عزبة زان شارع الهرم بجوار دارلي ، الجيزة ، مصر 4
1 Giza
Giza, Egypt

Patient Name:	**MERVAT MOUSTAFA ALI ABDALLA**	Gender:	**Female**
MRN:	**2020142884**	Ordered By:	**Eman Sobhy Adrawos Mahgoub**
DOB:	**16/03/1961** Age: **65 Y 0 M 21 D**	Order ID:	**3723390**

1. Stable L4 upper vertebral end plate sclerotic lesion.
2. Left femoral neck irregular cortical outlines.
3. Mild dorsolumbar spondylosis.
4. Bilateral sacroiliitis is noted
5. Still seen bilateral iliac, sacral and right femoral head/neck sclerotic foci.

Opinion:
** Status follow-up showing:
- Rather stationary course since the last study dated January 2026 with no newly developed lesions.
Approved by: Reham Anwar, FEBR, MD
Date:06/04/2026 Time:09:02

Reported and electronically endorsed by:
Reham Anwer Mohamed Fareid

Reported on: 06/04/2026 12:51:00 PM

CT Scan
[[DOCTYPE:CT_SCAN]]

بهية
مركز بهية للاكتشاف المبكر
وعلاج سرطان الثديات

Baheya

T 16462
F 16462
W info@baheya.org
E www.baheya.org
شارع طريبة أرل شارع الهرم بعمارة دارلي . الجيزة . مصر 4
شارع طريبة أرل شارع الهرم بعمارة دارلي . الجيزة . مصر 4
1 Giza
Giza, Egypt

Patient Name:	**MERVAT MOUSTAFA ALI ABDALLA**	Gender:	**Female**
MRN:	**2020142884**	Ordered By:	**eman abdelnaby amin elsayed**
DOB:	**16/03/1961** Age: **64 Y 9 M 27 D**	Order ID:	**3576716**

CT ABDOMEN AND PELVIS

Procedure Date: 10/01/2026 03:04:00 PM

Report:

Report:
NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS

**Reported case of breast cancer underwent surgical intervention.
**Reviewing the previous study dated 24/09/2025 the current study shows:

1- CHEST:
- Evidence of right mastectomy with still no sizable operative bed recurrent lesions. Few right axillary subcentimetric level I lymph nodes are still seen.
- No gross interval changes regarding the previously noted:
1. Few bilateral prominent supraclavicular subcentimetric lymph nodes.
2. Few bilateral tiny pulmonary nodules; the largest is still seen at the middle lobe, still measures about 0.5 cm in diameter.
3. Middle lung lobe and lingular parenchymal bands.
4. Prominent right pericardial fat pad.
5. Prominent mediastinal lymph nodes.
- No gross cardiomegaly.
- Mildly patulous thoracic esophagus.
- Aberrant right subclavian artery (variant).
- No pleural or pericardial effusion detected.
- Still noted left thyroid lobe small calcified nodules

2- ABDOMEN AND PELVIS:
- The liver is mildly enlarged in size. No definite hepatic focal lesions on non contrast basis.
- No intra-hepatic or extra-hepatic biliary radicles dilatation on non contrast basis.
- Normal non contrast appearance of the pancreas, spleen, kidneys and adrenals.
- Still noted:
1. Bulky uterus, for US assessment.
2. Small fat containing umbilical hernia.
3. Mild Aortic atheromatous calcification.
- Regular filling of the urinary bladder showing smooth outline with no sizable masses or diverticular outpouching.
- No abdominal or pelvic masses or collections.
The revised bone window revealed:

CT Scan
[[DOCTYPE:CT_SCAN]]

بهية
مركز بهية للاكتشاف المبكر
وعلاج سرطان الثدي

Bahaya

T 16002
F 16002
W info@bahaya.org
E www.bahaya.org
شارع عزبة أرض شارع الهرم بجوار نادي ، الجزة ، مصر 4
شارع عزبة أرض شارع الهرم بجوار نادي ، الجزة ، مصر 4
1 Giza
Giza, Egypt

Patient Name:	**MERVAT MOUSTAFA ALI ABDALLA**	Gender:	**Female**
MRN:	**2020142884**	Ordered By:	**eman abdelnaby amin elsayed**
DOB:	**16/03/1961** Age: **64 Y 9 M 27 D**	Order ID:	**3576716**

1. Stable L4 upper vertebral end plate sclerotic lesion.
2. Left femoral neck irregular cortical outlines.
3. Mild dorsolumbar spondylosis.
4. Bilateral sacroiliitis is noted
5. Still seen bilateral iliac and sacral sclerotic foci.

Opinion:
** Status follow-up showing:
- Rather stationary course since the last study dated 24/09/2025.
Approved by: Mennatallah Mohamed, MD
Date:10/01/2026 Time:19:11

Reported and electronically endorsed by:
Mennallah Mohamed Hanafy Hassan

Reported on: 11/01/2026 02:44:00 PM

CT Scan
[[DOCTYPE:CT_SCAN]]

بهية
مركز بهية للاكتشاف المبكر
وعلاج سرطان السيدات

Baheya
T 16602
F 16602
W info@baheya.org
E www.baheya.org
شارع جزيرة أول شارع قهوم بعمارة دكتور، الحوة، مصر 4
شارع جزيرة أول شارع قهوم بعمارة دكتور، الحوة، مصر 4
1 Giza
Giza, Egypt

Patient Name:	**MERVAT MOUSTAFA ALI ABDALLA**	Gender:	**Female**
MRN:	**2020142884**	Ordered By:	**Nermeen Mostafa Mahmoud Ahmed**
DOB:	**16/03/1961** Age: **65 Y 0 M 21 D**	Order ID:	**3454362**

CT ABDOMEN AND PELVIS

Procedure Date: 24/09/2025 09:20:00 AM

Report:

Report:
NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS

**Reported case of breast cancer underwent surgical intervention.
**As compared to the previous study dated 27/01/2025, the current study shows:

1- CHEST:
- Evidence of right mastectomy with no sizable operative bed recurrent lesions. Few right axillary subcentimetric level I lymph nodes are still seen.
- Rather, stationary course regarding the previously noted:
1. Few bilateral prominent supraclavicular subcentimetric lymph nodes.
2. Few bilateral tiny pulmonary nodules; the largest is still seen at the middle lobe, still measures about 0.5 cm in diameter.
3. Middle lung lobe and lingular parenchymal bands.
4. Prominent right pericardial fat pad.
5. Prominent mediastinal lymph nodes.
- No gross cardiomegaly.
- Mildly patulous thoracic esophagus.
- Aberrant right subclavian artery (variant).
- No pleural or pericardial effusion detected.
- Still, noted left thyroid lobe small calcified nodules

2- ABDOMEN AND PELVIS:
- The liver is mildly enlarged in size. No definite hepatic focal lesions on non contrast basis.
- No intra-hepatic or extra-hepatic biliary radicles dilatation on non contrast basis.
- Normal non contrast appearance of the pancreas, spleen, kidneys and adrenals.
- Still noted:
1. Bulky uterus, for US assessment.
2. Small fat containing umbilical hernia.
3. Mild Aortic atheromatous calcification.
- Regular filling of the urinary bladder showing smooth outline with no sizable masses or diverticular outpouching.
- No abdominal or pelvic masses or collections.
The revised bone window revealed:

CT Scan
[[DOCTYPE:CT_SCAN]]

مركز بهية للاكتشاف المبكر
وعلاج سرطان الثدي

Baheya

T 16632
F 16632
W info@baheya.org
E www.baheya.org
شارع هدى أول شارع الهرم بجوار نادي الجزيرة بمصر ٨
شارع هدى أول شارع الهرم بجوار نادي الجزيرة بمصر ٨
1 Giza
Giza, Egypt

Patient Name:	**MERVAT MOUSTAFA ALI ABDALLA**	Gender:	**Female**
MRN:	**2020142884**	Ordered By:	**Nermeen Mostafa Mahmoud Ahmed**
DOB:	**16/03/1961** Age: **65 Y 0 M 21 D**	Order ID:	**3454362**

1. Stable L4 upper vertebral end plate sclerotic lesion.
2. Mild dorsolumbar spondylosis.
3. Bilateral sacroiliitis is noted
4. Still seen bilateral iliac and sacral sclerotic foci.

Opinion:
** Status follow-up showing:
- Rather stationary course compared to the previous study.

Approved by: Marwa Ahmed, MD
Date:24/09/2025 Time:10:35

Reported and electronically endorsed by:
Marwa ahmed ahmed abo elhassan

Reported on: 24/09/2025 11:36:00 AM

CT Scan
[[DOCTYPE:CT_SCAN]]